

Practical Pointers Guide

Managing Daily Life with Advancing Parkinson's



About This Guide

This guide focuses on everyday activities such as dressing, bathing, eating and sleeping. It offers practical ideas for adapting daily routines as Parkinson's symptoms change over time.

This guide can help you:

- Find new ways to approach everyday activities
- Learn about tools and aids that may help with daily tasks
- Make your home safer and more comfortable
- Explore support from professionals, family and community
- Learn practical ways care partners can support daily routines

This guide offers general information. Talk to your care team for guidance specific to you.

Parkinson's Foundation Resources

As you explore this guide, remember that you are not alone. Our Helpline can answer questions about daily living and connect you to helpful resources. Call **1-800-4PD-INFO (1-800-473-4636)** or email Helpline@Parkinson.org to reach a trained specialist.

Acknowledgments

This guide was created in collaboration with specialists, members of the Parkinson's community and Parkinson's Foundation staff.

We would like to extend a special thank you to the following Parkinson's specialists for their contributions:

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How to Use This Guide

Follow this path to get the most from this guide:



Start with symptoms

Go to *The Symptom Iceberg* to identify symptoms that affect daily life. This helps you recognize what may be getting in the way.



Identify your supports

Go to *Build Your Support Team* to see who can help and how. Support may include your care team, family, friends and others.



Focus on daily activities

Go to the sections on everyday activities. Start with what matters most right now, or read through them all.

Each section includes:

- Self-check
- Practical tips
- Tools and supports
- Care partner tips

The tips and tools in this guide come from the lived experience of people with Parkinson's and care partners, along with input from specialists. Not everything will fit your needs. For some changes, it may help to check in with your care team for individualized guidance, especially if something feels unsafe or involves added cost.

The Symptom Iceberg

Parkinson's can cause many different symptoms. Some — such as tremor, stiffness or slowed movement — are easier to see and are often described as the tip of the iceberg. Others may be less visible but can still affect daily activities and quality of life. As Parkinson's progresses, symptoms may change or new symptoms may appear.

Learn about these symptoms to help you notice changes and identify topics to discuss with your care team.

More visible symptoms

- Tremor, stiffness, slower movement
- Balance issues
- Walking changes

Less visible symptoms

- Fatigue or low energy
- Sleep problems
- Anxiety, depression and apathy (low motivation)
- Changes in thinking or focus
- Constipation
- Bladder issues
- Pain or discomfort
- Swallowing changes
- Low blood pressure (dizziness when standing)
- Breathing issues



Learn more about Parkinson's symptoms at [Parkinson.org/MovementSymptoms](https://www.parkinson.org/MovementSymptoms) and [Parkinson.org/NonMovementSymptoms](https://www.parkinson.org/NonMovementSymptoms).

Build Your Support Team

Building a strong support team is important at every stage of Parkinson's and can become even more essential as daily tasks become harder to manage on your own.

1

Work with your care team

Regular appointments with your Parkinson's care team can help monitor symptom changes, address daily challenges and explore treatment options.

Key actions

- Notice and track changes in symptoms. This is easier to do when you write things down as they happen or keep a daily log.
- Write down your top questions, concerns and goals before appointments.
- Share examples of how symptoms or other concerns affect daily activities. Some symptoms may not come up unless you mention them.
- Ask what options may help, including medication adjustment or referrals to specialists.

Medication reminder

Missing or delaying doses can affect how you feel, move and function during the day. Let your care team know if you miss doses or have trouble following your medication schedule.

Download the Steps to Prepare for a Parkinson's Appointment worksheet at [Parkinson.org/OptimizingCare](https://parkinson.org/optimizingcare).

2

Include rehabilitation therapists

Physical, occupational and speech therapists are an important part of Parkinson's care. They can help improve daily function, support safety and adapt routines as needs change. They can also teach care partners safe ways to help.

Physical Therapists

Focus on improving movement and mobility. They may help with:

- Walking, balance and fall prevention
- Exercises to maintain strength and flexibility
- Strategies for freezing and turning
- Safe movement when getting in and out chairs, bed or car

Occupational Therapists

Focus on everyday activities. They may help with:

- Daily activities such as bathing, dressing, and eating
- Simplifying routines and managing fatigue
- Cognitive skills such as attention, planning and completing tasks
- Identifying and using adaptive equipment, such as special utensils
- Making home modifications to improve safety and independence

Speech Therapists

Focus on communication, swallowing and thinking changes. They may help with:

- Making speech clearer and easier to hear
- Ways to communicate more effectively
- Checking swallowing and suggesting safer ways to eat or drink
- Managing changes in memory and finding words

Ask your Parkinson's care team about referrals to physical, occupational or speech therapists with Parkinson's experience. For help finding specialists or local resources, contact the Parkinson's Foundation Helpline at **1-800-4PD-INFO (1-800-473-4636)** or **Helpline@Parkinson.org**.

3**Expand your support circle**

Managing daily activities with Parkinson's often works best as a shared effort. Looking beyond your immediate household can help with transportation, daily tasks, emotional support and hands-on care as needs change. You are not alone.

Support may come from many places, including:

- Family members or care partners
- Friends, neighbors or community groups
- Support groups, including virtual options
- Personal care aides who provide paid, hands-on help with daily activities

See the Resources section at the end of this guide to explore more ways to build support.

Everyday Movement

Parkinson's can affect the basic movements you rely on throughout the day, including sitting, standing and walking. These movements are used in activities like dressing, bathing and getting in and out of bed.

This chapter introduces movement strategies that can help make everyday activities easier and safer. You can apply these ideas throughout the rest of the guide.

Movement self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off anything that applies to you:



Feeling unsteady when walking or turning



Taking small steps, shuffling or walking faster than I mean to



Freezing or feeling stuck while walking



Trouble lowering into or standing up from a chair



Tripping or falling



Trouble getting in or out of the car



Spending most of the day sitting or lying down

What other challenges have you noticed?

Top Tips for Safer Movement

Use these tips as a guide and adjust based on your needs.



Take your time

Rushing can lead to trips and falls.



Create a safe space

Remove clutter, cords and throw rugs. Watch for pets that may move quickly underfoot. Make sure spaces are well lit, especially at night.



Avoid multitasking

Dividing attention between walking and other tasks can affect balance.



Improve your stability

Grab bars, handrails and well-fitting shoes can improve balance and reduce fall risk.



Manage your energy

Plan activities for times when your medication is working well or when you feel more rested. Take breaks when needed.



Work on strength and balance

A physical therapist can recommend exercises to make everyday movement, such as walking or climbing stairs, safer and easier.

Care Partner Safety Tips

Be flexible. Movement and daily activities can vary throughout the day and from one day to the next. Work together to adjust plans and allow extra time.

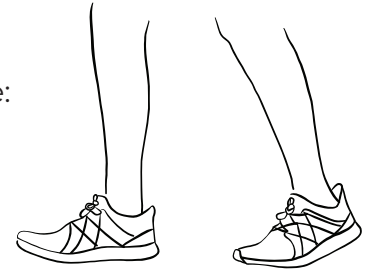
Walking, Freezing and Falls

Parkinson's can cause walking changes such as shuffling, difficulty turning or freezing (when feet feel stuck). A physical therapist can help address these challenges and support safer movement.

Tips for walking and turning

A physical therapist can help you practice strategies like these:

- Take bigger steps to reduce shuffling
- Lift your feet fully off the floor when stepping
- Look ahead while walking
- Turn using several small steps instead of twisting your body



Care Partner Tip

Walk beside the person you are helping so you can match their pace and offer support. Use a calm, reassuring voice to reduce anxiety.

If freezing happens

- Stop, stand tall, shift your weight and step forward
- Focus on stepping over a visual target, such as a line or object
- Use counting, clapping or music to create rhythm and guide movement

Tips to prepare for a fall

Even with precautions, falls can still happen. Being prepared can help you get up safely or reach someone for help if you cannot get up on your own.

- Practice safe ways to get up after a fall with a physical therapist
- Wear a medical ID and a fall detection device
- Keep a phone or voice-activated call system and emergency contacts nearby
- Call for help if you cannot get up safely on your own to avoid further injury

How to Choose a Mobility Aid

Mobility aids such as walkers or wheelchairs can make movement easier when walking feels unsteady or tiring. Insurance may cover these devices with a doctor's prescription. A physical therapist can help you choose the right aid, adjust it to fit and show you how to use it.

Choosing a walker

Different walker styles offer different levels of support:

- **2-wheeled walker:** Provides steady support for balance. A tray or bag can help carry items.
- **4-wheeled walker (rollator):** Turns more easily and often includes a seat and basket but may be less stable than a 2-wheeled walker.
- **Walker without wheels:** Must be lifted to move and may increase the risk of losing balance.



Choosing a wheelchair

- **Folding wheelchair:** Lightweight and easier to store or transport but often requires someone to push it.
- **Everyday wheelchair:** May include features such as back support, cushions and footrests for comfort and positioning.

Getting comfortable with a mobility aid

- It may take time to adjust to using a mobility aid.
- Many people find the right aid helps them stay active and move around more comfortably.

Care Partner Tip

Adjusting to new habits is often hard. Offering gentle reminders can help support consistent use of mobility aids.

Sitting and Standing

Standing up or sitting down may take more effort as Parkinson's symptoms change.

What may help

- Use chairs with armrests and higher seats
- Before sitting, move back until your legs touch the chair
- Before standing, scoot to the edge of the seat, lean forward and push up with your hands
- Helpful tools may include lift chairs, stand-assist rails or gait belts

Care Partner Tip

When assisting with standing use clear cues like “nose over toes.”

Getting In and Out of Cars

Getting in and out of a car can be challenging because it combines stepping, turning and sitting or standing, often in tight or uneven spaces.

What may help

- Park where there is space to open the door fully and step onto a flat surface
- Take your time and pause before standing or sitting if you feel unsteady
- Sit first, then bring your legs into the car
- Use the door frame or seat for support when getting in or out of the car
- Lock the wheels of walkers or wheelchairs before moving in or out



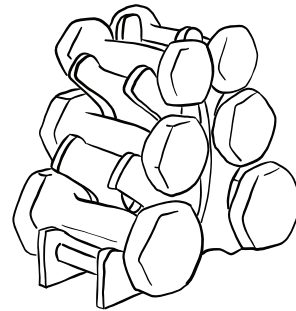
Movement Matters

Regular movement supports strength, flexibility and overall well-being. For Parkinson's, exercise can also help manage symptoms.

As symptoms change, the way someone stays active may also change. Continuing to move in ways that work for you can still support strength and well-being.

Aim for about 30 minutes of activity each day if possible. This can be broken into shorter sessions. Examples may include:

- Walking, cycling or water exercise
- Chair-based exercises if standing is difficult
- Gentle stretching or range-of-motion exercises
- Small movements in bed or a chair to help circulation and comfort



Care Partner Tip

Parkinson's affects brain chemicals that help control movement and can also affect mood and motivation. This may make it harder to get started with activities. Enjoyable activities and moving together can make it easier to stay active.

Helpful Resources

Learn more about movement, balance and fall prevention at [Parkinson.org/Library](https://www.parkinson.org/Library).

Watch CareMAP videos for practical tools and tips designed for care partners at [Parkinson.org/CareMAP](https://www.parkinson.org/CareMAP). Look for topics such as “changes around the house,” “movement and falls” and “travel and transportation.”

Getting Dressed

Getting dressed involves balance, coordination, planning and fine motor control. With Parkinson's, changes such as slower movement, stiffness or fatigue can make these tasks take longer or require more effort. Some parts of dressing, like putting on socks, fastening buttons or choosing clothes, may require more time, concentration or support.

This chapter shares practical strategies and tools to help make dressing easier, reduce frustration and lower the risk of falls.

Dressing self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off any that apply to you:



Buttons, zippers or other fasteners are hard to manage



Trouble bending or reaching to put on socks, pants or shoes



Dressing takes much longer than it used to



Difficulty choosing or organizing clothing



Fatigue makes dressing harder



Feeling unsteady when dressing or needing to sit down



Dizziness, falls or near falls when getting dressed

What other challenges have you noticed?

Top Tips for Easier Dressing

Use these tips as a guide and adjust based on your needs.



Plan ahead

Lay out clothes the night before or hang outfits in the closet



Build strength

A physical therapist can help strengthen core and leg muscles



Dress when you have more energy

Usually after your morning dose has taken effect



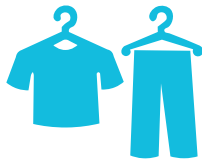
Warm up first

Light movement or stretching can help you feel more flexible



Consider sitting to dress

Helps with balance and reduces fatigue



Keep clothes within easy reach

Store everyday clothes on lower shelves or hang them where they are easy to reach

Care Partner Tip

If you are giving more hands-on dressing assistance, it can be physically demanding. An occupational therapist can show you ways to protect your body.

Dressing Tools

Sometimes a tool can make dressing faster or easier. Other times, simplifying clothing may be the best approach. Use what works for you. An occupational therapist can help you find the right tools or clothing for you.

Using a chair as a dressing tool

A sturdy chair with armrests can be helpful during dressing, especially if:

- A little extra support is helpful while standing
- Sitting feels safer when feeling tired, dizzy or off balance
- Lifting a foot to put on socks, pants or shoes is easier while seated
- A short rest during dressing is needed

Avoid

Sitting on the edge of a bed to get dressed. Beds are soft and can shift, which may make balance harder.

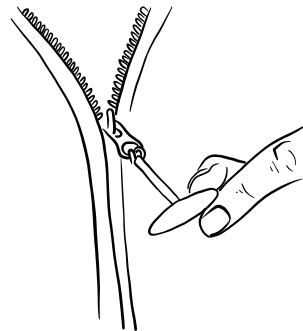
Long-handled shoehorns

Help you slide into shoes without bending forward



Zipper pulls

Add a larger loop or handle to a zipper to make it easier to grip



Care Partner Tip

Clothing choices often reflect personal style. If you're helping pick out outfits, look for options that are easier to put on and still feel like something the person would normally wear.

Easier Clothing Options

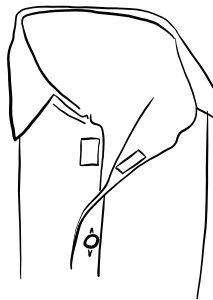
Front-opening shirts

Shirts that open in front are often easier to manage than pullovers



Simple closures

Magnetic closures, Velcro or elastic waistbands can replace small buttons or zipper pulls



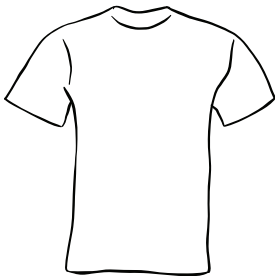
Roomy, soft clothing

Loose necklines and pant legs are easier to get into and move in



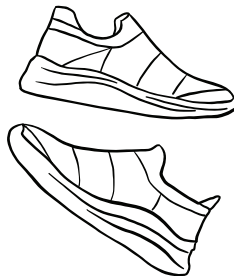
Stretchy fabric

Clothing with stretch can be easier to pull on and adjust



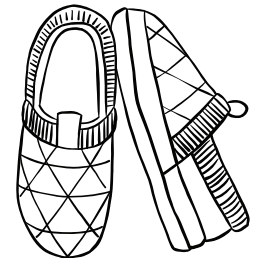
No-tie footwear

Slip-on styles or no-tie laces can make it quicker to get shoes on and head out the door



Slippers with backs

Stay on better than open-back slippers and may help prevent trips or falls



Helpful Resources

Learn more about dressing and clothing adaptations at [Parkinson.org/Dressing](https://www.parkinson.org/Dressing).

Watch CareMAP video on dressing for practical tools and tips designed for care partners at [Parkinson.org/CareMAP](https://www.parkinson.org/CareMAP).

Personal Care

Bathing, grooming and using the toilet are everyday tasks that may become more complicated with Parkinson's. The bathroom can also be one of the highest-risk areas in the home for slips and falls. Some people may also notice bladder or bowel changes that affect personal care.

This chapter shares practical tips and tools to help make personal care safer and easier to manage.

Personal care self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off any that apply to you:



Falls or concerns about falling in the bathroom



Trouble using grooming tools (razor, toothbrush, hairbrush)



Skipping or forgetting grooming or hygiene tasks



Difficulty reaching all areas for washing or grooming



Feeling unsteady getting into or out of the shower or tub



Not feeling fully clean after bathing



Difficulty sitting down or standing up from the toilet



Bladder or bowel changes (urgency, leaks or constipation)



Difficulty cleaning after using the toilet

What other challenges have you noticed?

Top Safety Tips in the Bathroom

Use these tips as a guide and adjust based on your needs.



Choose stable supports

Grab bars, shower chairs and raised toilet seats help prevent falls when standing or sitting



Keep floors dry and use non-slip mats

Reduces the risk of slips and falls



Sit when you need to

Brushing teeth, washing or shaving can be safer seated if you're feeling unsteady on your feet



Add nightlights or motion lights

Helps you navigate safely in the dark if you need to get up to use the bathroom



Keep toiletries and towels within easy reach

Avoids overreaching or twisting, which can cause falls



Have a way to call for help

Leave the door slightly open, bring a phone with voice commands or use an alert system

Care Partner Tip

Bathroom routines may take more time. A slower pace can help reduce the risk of slips, falls and frustration.

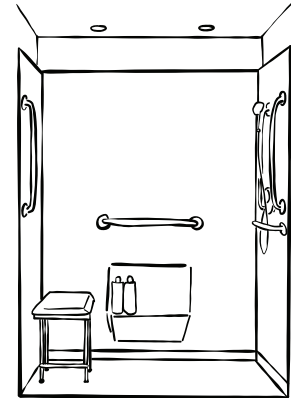
Personal Care Tips

Grooming

- Break tasks into steps (wash your face, rest, brush teeth or shave).
- Use reminders such as checklists or large notes.
- Sip water, chew sugar-free gum or use mouthwash or oral swabs for dry mouth.

Bathing

- Use an easy-entry shower if available. Walk-in or no-lip showers reduce fall risk.
- Use warm water. Water hotter than 120°F can irritate skin and affect blood pressure.
- Dry skin completely after bathing to help prevent irritation or sores.



Care Partner Tip

If you're helping with bathing, a sponge bath may be easier on days when a full shower or bath is tiring. Expose only the area being washed to help maintain warmth and privacy.



Using the Toilet

Sit and stand with care

- Move back until your legs touch the toilet before sitting.
- Stand slowly. If you feel dizzy or unsteady, sit back down.
- Use a raised toilet seat or grab bars if needed to make sitting and standing easier.

Manage urgency and constipation

- Clothing that is easy to remove may help when bathroom trips feel urgent.
- Plan ahead for bathroom needs. Use the toilet before leaving home or at regular times if helpful and know where bathrooms are when you are out.
- Drink fluids, eat fiber-rich foods such as oatmeal, ripe pears or cooked vegetables and stay active to help keep bowel movements regular.
- Talk to your care team if urgency or constipation is ongoing or difficult to manage. Treatments may include diet changes, medications or pelvic floor therapy.

Plan for nighttime bathroom use

- Keep a clear, well-lit path to the bathroom, especially at night.
- Consider a urinal or bedside commode to reduce nighttime trips and fall risk.

Care Partner Tip

Bathroom care can feel uncomfortable for both of you. Talk ahead of time about what help feels acceptable and when outside support may be needed.

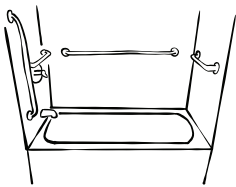
Bathroom Tools

An occupational therapist can recommend bathroom tools and equipment based on your daily routines and show how to use them safely.

Supports

Grab Bars

Install securely near the shower, tub and toilet for support



Shower chair or tub transfer bench

Provides seated support and may reduce slips and fatigue



Raised toilet seat or commode frame

Makes sitting and standing easier



Grip and Reach

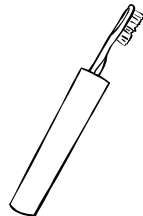
Long-handled brushes, combs or back scrubbers

Help reduce bending and support balance



Foam tubing on toothbrushes, razors or brushes

Makes items easier to hold



Electric toothbrushes, flossers or shavers

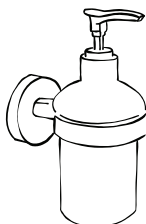
May be easier to grip and use



Hygiene

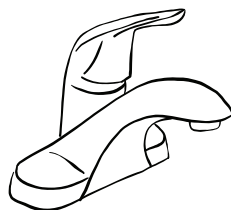
Liquid soap pumps

Easier to use with wet hands



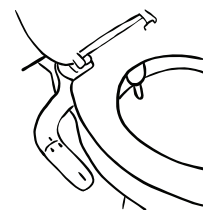
Lever-handled or motion-sensor faucets

Take less effort to turn on and off



Bidets or disposable wipes

Provide gentler cleaning



Managing Bladder Leaks

As Parkinson's progresses, bladder issues such as urgency or leaks may become more common. This is because Parkinson's affects the muscles and nerves that control the bladder. These issues can be challenging, but there are ways to manage them.

Talk with your doctor

Ask about treatment options or a referral to a pelvic floor specialist, urologist or urogynecologist.

Use absorbent products if needed

Pads or disposable underwear can help manage leaks and boost confidence. Disposable underwear may be especially helpful when away from home.

Protect furniture or bedding

Underpads can reduce worry and make cleanup easier.

Reduce nighttime bathroom trips

Limit drinks in the evening if recommended and consider a urinal, bedpan or bedside commode. Some drinks, like caffeine or alcohol, may increase nighttime urination.

Some people drink less water because they worry about leaking or falling when getting up at night. It's important to stay hydrated — usually about 6–8 glasses a day, unless your doctor recommends otherwise.

Find support for incontinence from the National Association for Continence at [Naafc.org](https://naafc.org).

Helpful Resources

Learn more about bladder issues, incontinence and at Parkinson.org/NonMovement.

Watch CareMAP video on personal care for practical tools and tips designed for care partners at Parkinson.org/CareMAP.

Mealtimes & Swallowing

Mealtimes can become one of the more challenging parts of the day as Parkinson's progresses. Using utensils or managing food may take more effort, and chewing or swallowing may need more time or attention. Appetite or weight may also change over time.

This chapter shares practical strategies and tools to help make mealtimes and swallowing easier and safer.

Mealtimes self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off any that apply to you:



Difficulty getting food from the plate to the mouth



Keeping food on the plate while eating



Feeling too fatigued to finish a meal



Food feeling stuck or taking more effort to swallow



Food or drink going down the wrong way (coughing or choking)



Reduced appetite



Losing weight without trying

What other challenges have you noticed?

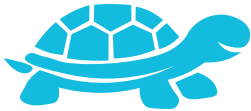
Top Tips for Safer, Easier Mealtimes

Use these tips as a guide and adjust based on your needs.



Support your body while eating or swallowing pills

Sit as upright as you can, even if you need to use cushions or wedges. This can support digestion and swallowing



Slow your pace

Small bites and sips, thorough chewing and taking your time while eating can make meals safer and less tiring



Eat during “on” times

Meals may feel easier when medication is working well and movement feels smoother



Choose foods that are easier to handle

Foods that stay together or can be picked up easily may be easier to handle



Reduce the chance of spills

Use a cup that feels steady, such as one with handles or a lid



Rinse and brush after meals

Protects teeth and gums and lowers the risk of lung infections for people with swallowing difficulties

Care Partner Tip

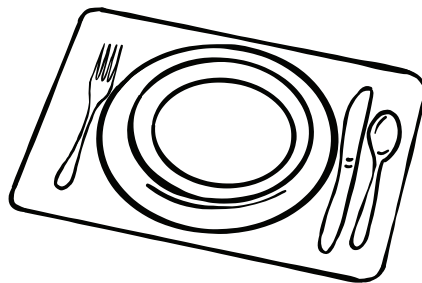
If you are helping with meals, allow extra time and avoid rushing. Reheating food or offering smaller portions may help if meals take longer. A calmer pace can reduce stress.

Mealtime Tools

Your needs and preferences are unique to you and may change over time. An occupational therapist, speech therapist or another member of your care team can help you find tools that work best for you.

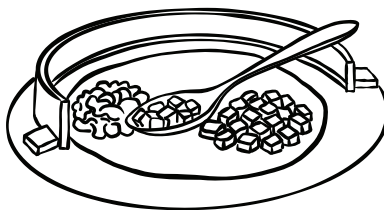
Nonslip placemats

Made from material like rubber or silicone to keep plates and bowls from sliding



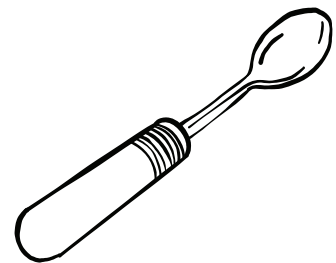
Plates with guards or raised edges

Help keep food on the plate and make it easier to scoop food onto a spoon or fork



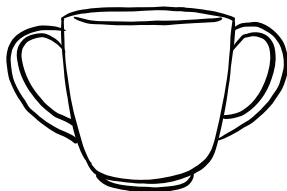
Easy-grip utensils

Spoons and forks with bigger handles or soft grips are easier to hold if hands shake or feel weak



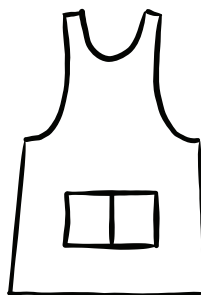
Two-handled cups

Offer steadier control when drinking, which can help with tremor or reduced hand strength



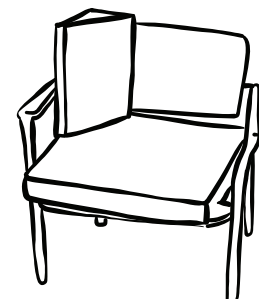
Protective clothing

Aprons, adult bibs or collar-tucked napkins can make meals less stressful



Postural supports

Cushions or wedge pillows help maintain upright posture, which improves swallowing



Care Partner Tip

Try new tools together and focus on what makes meals easier and more enjoyable for both of you.

Meal Preparation

The same symptoms that affect other daily activities can make meal preparation harder or unsafe. These may include movement changes, low blood pressure or fatigue. Thinking changes can also play a role.

If safety is a concern or preparing meals is challenging, talk with your care team for guidance. Some tasks may be doable with a different approach or a bit of support while others may be better to hand off. Work with an occupational therapist for strategies and tools to help. You may also try:

- Tools with larger, easier to grip handles
- Sitting while preparing ingredients or cooking to reduce fatigue
- Using pre cut ingredients or tools that reduce the need for chopping
- Non slip mats to keep items steady
- Breaking tasks into smaller, simpler steps
- Gathering all ingredients in one place before you start

Caution

Cooking can involve risks like falls, cuts, burns or leaving the stove on. Being aware of these hazards can help you decide what feels safe and when support may help.

Dining Out

Eating at restaurants can bring a few challenges. A few practical choices can make meals easier.

Consider the following:

- Look for restaurants with table service and make a reservation when possible.
- Consider access (close parking, no stairs, clear paths and accessible restrooms).
- Ask for a bowl instead of a flat plate and for sandwiches cut in half.
- A spoon may be easier to use than a fork.

Easier Foods

- Sandwiches or wraps
- Short pasta shapes
- Mashed potatoes
- Thick soups or stews

Harder Foods

- Spaghetti
- Peas or corn
- Leafy salads
- Foods that crumble

Are you Having Trouble Swallowing?

Swallowing can change with Parkinson's and may become more difficult over time. This is sometimes called dysphagia, which means trouble swallowing.

Signs can include:

- Trouble swallowing pills or food
- Food feeling stuck in the throat
- Coughing during or after meals or drinks
- Drooling or a “gurgly” voice
- Unplanned weight loss
- Fever

If you notice these signs, ask your care team for a referral to a speech-language pathologist (SLP) for a swallowing evaluation. This may involve a barium swallow or fiberoptic evaluation. Afterward, the SLP may suggest strategies like:

- Positioning or seating supports to help you stay upright while eating
- Eating without distractions
- Taking small bites, eating slowly and chewing thoroughly
- Avoiding dry, crumbly foods like crackers or chips
- Trying thickened liquids or smoothies if recommended

Swallowing problems can lead to aspiration, when food or liquid enters the lungs. This can happen without coughing (called silent aspiration) and may increase the risk of pneumonia, a serious lung infection that can be life-threatening.

Swallowing pills

If swallowing pills is difficult, talk to your care team. They may suggest:

- Taking pills with soft foods like applesauce or yogurt
- Crushing immediate release tablets or opening certain capsules (including Rytary) and mixing with soft food
- Explore pump medications or other delivery methods

Swallowing is especially important in the hospital, where routines and medications may change. Order your free Hospital Safety Guide at

[Parkinson.org/HospitalSafety](https://parkinson.org/HospitalSafety).

Tips for Managing Weight Loss

Unplanned weight loss is common in Parkinson's. It can be linked to swallowing changes, medication side effects, low appetite, fatigue, loss of smell or difficulty preparing food. A registered dietitian can help with meal planning.

- Boost flavor — Herbs, spices or citrus can make food more appealing
- Cook with whole milk, cream or coconut milk
- Add butter, nut butters, healthy oils or avocado to meals
- Snack between meals
- Eat smaller, more frequent meals (especially helpful with nausea or bloating)
- Sprinkle flax, chia or hemp seeds into foods
- Choose protein-rich foods like eggs, yogurt, beans, tofu or chicken to support muscle health

Check with your doctor before using high-calorie supplements or making major diet changes.

If you take levodopa: Eating protein too close to a dose can lessen its effect for some people. Common timing guidelines suggest eating about 60 minutes before or 30–60 minutes after medication.

Example: noon dose → eat by 11 a.m. or after 12:30 p.m.

Helpful Resources

- Learn more about swallowing [Parkinson.org/NonMovement](https://www.parkinson.org/NonMovement).
- Read the “Speech & Swallowing” fact sheet at [Parkinson.org/Library](https://www.parkinson.org/Library).
- Watch the CareMAP videos on mealtimes and swallowing for practical tools and tips designed for care partners at [Parkinson.org/CareMAP](https://www.parkinson.org/CareMAP).

Sleep & Moving in Bed

Sleep plays an important role in how you feel and function during the day. With Parkinson's, sleep can be affected in different ways and often changes over time. One common pattern is waking during the night or too early, sometimes called fragmented sleep.

Other challenges may include daytime sleepiness, medication-related sleep changes or difficulty moving in bed or getting up safely at night.

This chapter shares practical tips and tools to help make personal care safer and easier to manage.

Personal care self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off any that apply to you:



Waking often during the night or too early



Feeling very sleepy or sleeping more during the day



Days and nights feel flipped



Difficulty getting comfortable or turning in bed



Trouble getting in or out of bed



Concerns about balance or falling at night



Acting out dreams or sudden movements during sleep



Skin irritation or soreness from time spent in bed



Sleep issues related to medication side effects

What other challenges have you noticed?

Top Tips for Better Sleep

Use these tips as a guide and adjust based on your needs.



Keep a regular sleep schedule

Going to bed and waking up at the same time each day supports a steady sleep pattern



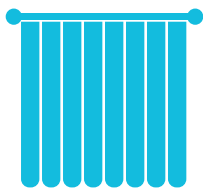
Keep naps short and earlier in the day

Long or late naps can make it harder to fall asleep at night



Wind down before bed

Choose calming activities and turn off screens about an hour before bedtime



Create a sleep-friendly bedroom

A cool, dark and quiet room promotes better sleep



Adjust the bed setup

Satin sheets and a firmer mattress can make it easier to turn and get comfortable in bed



Talk to your doctor about sleep changes

Sleep problems can have many causes, including movement symptoms returning during the night and sleep disorders

Care Partner Tip

Sleep changes can affect both of you. Talk and share concerns with the care team. If you share a bed, some people use separate blankets or choose separate sleeping arrangements.

Feeling very sleepy during the day?

Even with good sleep habits, feeling very sleepy during the day is common in Parkinson's and may become more frequent or harder to manage over time. You may find yourself napping more, having trouble staying awake or feeling like your days and nights are mixed up.

This can happen for different reasons, including:

- Broken or interrupted sleep at night
- Medications used to treat Parkinson's, especially dopamine agonists such as pramipexole, ropinirole or rotigotine, as well as other medications that can cause drowsiness, such as those for sleep, anxiety or mood
- Changes in the body's sleep-wake cycle (your internal clock), which can shift sleep toward the daytime and wakefulness at night
- Changes in sleep with aging, such as lighter or more interrupted sleep at night

If sleepiness continues, it may help to look more closely at your daily routine. The body's sleep-wake cycle is influenced by daily habits. Getting light during the day and staying active help signal the brain to stay alert, while long or late naps or caffeine later in the day can make it harder to sleep well at night and increase daytime sleepiness. Noting sleep times, naps and caffeine in a sleep log can help identify patterns.

When to talk with your doctor

If sleepiness is frequent, comes on suddenly or affects your daily activities, your doctor can help look at possible causes and next steps. This may include adjusting medications, looking more closely at nighttime sleep or evaluating sleep conditions such as sleep apnea. In some cases, other treatments may be considered to help with daytime alertness.

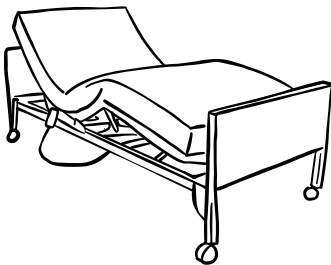


Tools to Help with Moving in Bed

A physical or occupational therapist can help identify tools and strategies to make turning, repositioning or sitting up in bed easier.

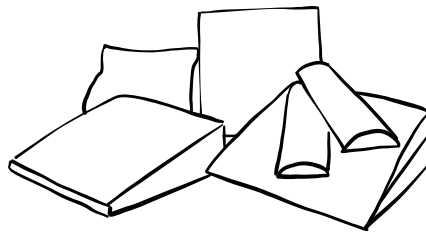
Adjustable beds

Let you raise or lower parts of the bed to help with comfort, sitting up and getting in and out



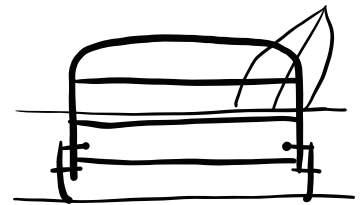
Cushions and pillows

Elevate the upper body so it's easier to start a roll or push into sitting



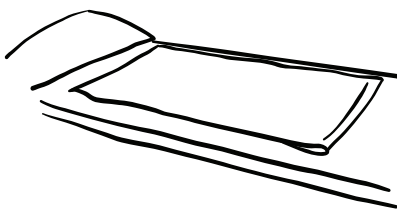
Bed rails or poles

Provide support when turning, sitting up or getting out of bed and help reduce the risk of falls



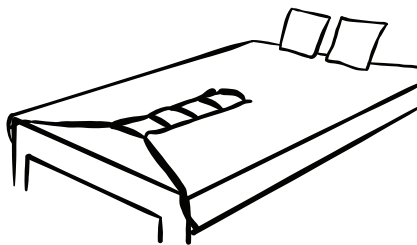
Slide sheets

Smooth fabric placed under the body to make turning or repositioning easier



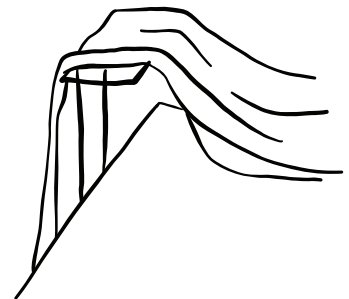
Bed ladders

Fabric or strap ladders that attach to the bed and help you pull into a sitting position



Blanket supports

Keep blankets off the feet to prevent tangling and make turning easier



Care Partner Tip

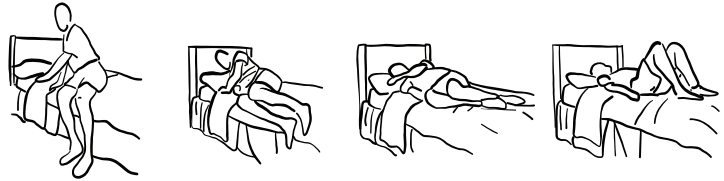
If you're helping with movement in bed, avoid pulling on arms or shoulders. Use pillows or a slide sheet to support safer movement. An occupational or physical therapist can show you techniques to make this easier and safer at home.

How to Move in Bed

A physical or occupational therapist can teach these techniques and how to practice them safely at home.

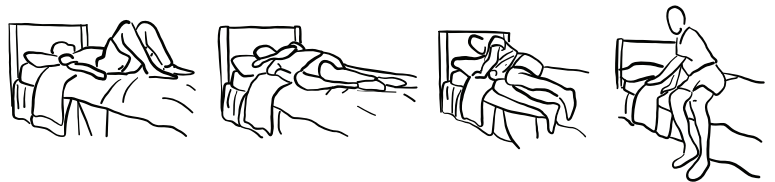
Getting into bed

- Sit on the edge of the bed
- Lower onto your side
- Bring your legs onto the bed
- Roll onto your back



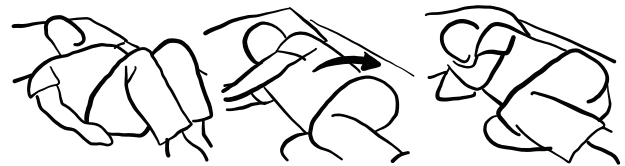
Getting out of bed

- Roll onto your side
- Let your legs move off the bed
- Push up to sit



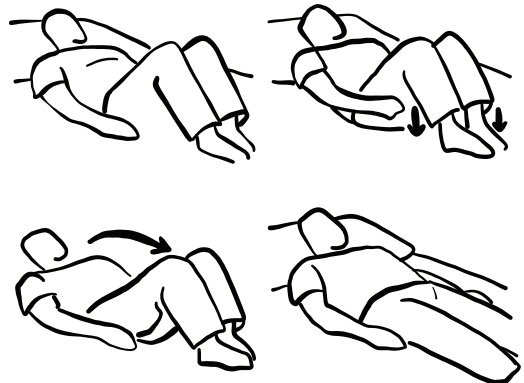
Turning over in bed

- Bend your knees and put your feet flat on the bed
- Turn your head and reach your arm
- Let your knees fall in the direction you want to roll



Scooting in bed

- Bend your knees and put your feet flat on the bed
- Push through your feet
- Shift your hips
- Finish in a comfortable position



Managing Nighttime Safety Challenges

Sleep issues can lead to falls or injury at night. These tips may help:

Getting up to use the bathroom

Keep the path clear and well lit. Sit at the bedside before standing and take your time. A bedside commode, urinal or sleeping closer to the bathroom may improve safety at night. If movement symptoms make it hard to get up, talk with your doctor about medication timing or longer-acting options.

Acting out dreams (REM Sleep Behavior Disorder)

Remove hazards around the bed to help prevent injury. Separate blankets or sleeping separately may also help. Talk with your doctor about treatments.

Risk of falling out of bed

Lower the bed or add padding beside it to help reduce injury. Talk with your care team before using bed rails.

Getting tangled in bedding or equipment

Keep bedding light. Fewer layers or separate blankets may help. If you use a CPAP machine, position tubing to avoid pulling or tangling.

Medication side effects

Some sleep medications can increase confusion or unsteadiness at night. Review your medications with your doctor.

Skin protection in bed

Spending more time in bed can increase the risk of skin breakdown. Change positions regularly. Check skin daily and talk with your care team about redness or positioning support.

Helpful Resources

Learn more about sleep in Parkinson's (including common sleep disorders) at [Parkinson.org/Sleep](https://www.parkinson.org/Sleep).

Watch CareMAP video on rest and sleep for practical tools and tips designed for care partners at [Parkinson.org/CareMAP](https://www.parkinson.org/CareMAP).

Thinking Changes & Daily Life

Parkinson's can affect thinking in different ways. For some, changes are mild and do not interfere with daily life. For others, they may become more noticeable over time and affect routines and independence.

This chapter focuses on practical strategies and tools to help manage thinking changes in daily life.

Thinking self-check

The list below highlights challenges that may develop or become more noticeable as Parkinson's symptoms change over time. Use it to help identify strategies in this chapter and guide conversations with your care team.



Check off any that apply to you:



Trouble focusing or slower thinking



Forgetting things or losing track of tasks



Difficulty planning, organizing or following steps



Trouble starting or finishing tasks



Losing track of conversations or words



Feeling overwhelmed in busy environments



Difficulty managing medications or schedules



Trouble with bills, paperwork or decisions



Changes in mood, motivation or behavior



Seeing or believing things that are not real

What other challenges have you noticed?

Thinking Changes and Daily Strategy

Understanding thinking changes in Parkinson's

- In Parkinson's, thinking changes often involve attention, planning and processing information. Memory changes may occur later or not at all.
- These changes usually develop slowly. Some are mild, while others may have a greater impact over time.
- In some cases, thinking changes can progress to dementia, a decline in thinking and memory that makes it harder to manage daily activities independently.
- Some people notice these changes themselves. In other situations, a care partner, family member or friend may notice them first.

The sections below describe common thinking challenges and ways to manage them. Use this information as a guide and adjust based on your needs.

Daily routines and planning

What might change

- Starting or staying focused long enough to finish daily routines (such as getting ready for the day) may become harder.
- Keeping track of plans, appointments or daily activities may become more difficult.

What can help

- Follow a simple, consistent daily routine (same order, same time).
- Use a large wall calendar or whiteboard and to-do lists to keep track of routines and daily tasks such as exercise, appointments and medication times.

Care Partner Tip

Support may include giving a cue for the next step, laying out items ahead of time or providing hands-on help. Alarms, reminders or other assistive technology can also help keep routines on track.

Depression, anxiety and apathy

Depression, anxiety and low motivation (apathy) can be part of Parkinson's. These symptoms can make it difficult to get started or follow through with daily routines and may sometimes look like thinking changes. Treatments such as therapy or medication can help. Learn more at [Parkinson.org/NonMovement](https://parkinson.org/NonMovement).

Managing medications

What might change

- It may be easier to miss doses or lose track of medication times.
- Medication schedules may feel confusing or harder to follow.

What can help

- Keep medication times consistent.
- Use a pill organizer or an automatic pill dispenser and set alarms or reminders.
- Link medications to daily activities (for example, after brushing teeth).
- Use a medication management app. Explore PD-related apps at [Parkinson.org/Apps](https://www.parkinson.org/apps).

Care Partner Tip

Signs that you may need to take a more active role include needing daily reminders, confusion about the schedule, mixing up medications or missed doses.

Household tasks and finances

What might change

- Tasks with several steps (such as cooking or laundry) may feel overwhelming.
- Managing finances may become more difficult.

What can help

- Break tasks into smaller steps and focus on one step at a time.
- Shift or share responsibility for more complex tasks if needed.

Care Partner Tip

It may help to work through tasks together, check in on progress or help manage certain responsibilities such as finances.

Driving and transportation

What might change

- Reacting quickly or managing busy driving situations may become more difficult.
- Following directions, judging distance or staying on route may become harder.

What can help

- Talk with your care team if you have concerns about safety.
- Plan ahead for other ways to get around (family, rides, community options).

Care Partner Tip

Driving can be a sensitive topic. If safety becomes a concern, involving family members or the care team may help support conversations.

Communication and social situations**What might change**

- It may take longer to respond, organize thoughts or find the right words during conversations.
- Following conversations may be harder, especially in busy or noisy environments.

What can help

- Let people know you may need a little more time to respond.
- Have conversations in smaller groups or one-on-one when possible.
- Reduce background noise when possible (for example, turn off TV or move to a quieter space).

Care Partner Tip

Conversations may move more slowly. Give extra time for the person to respond. Try not to interrupt. This can reduce frustration and make conversations easier.

Medical decisions and appointments**What might change**

- Filling out forms, taking in new information or making decisions can take more time or may require help.
- Keeping track of details from appointments may be more challenging.

What can help

- Bring a care partner, family member or friend to help listen, take notes and ask questions.
- Ask for written instructions or summaries when possible.
- Review appointment notes and outline next steps.

Care Partner Tip

Work together to decide the most important things to talk about at the appointment. Write them down. Bring the list with you so you can have a clear conversation with the healthcare team.

Advanced Thinking Changes in Parkinson's

In some cases, thinking changes in Parkinson's can progress to dementia. These changes can be difficult for everyone involved. Independence may be affected, and relationships may change as roles and responsibilities shift. Frustration or a loss of confidence often accompany these changes.

Other factors beyond Parkinson's may also affect dementia risk, including:

- Untreated hearing and vision problems
- Depression and social isolation
- Conditions such as high blood pressure, diabetes and stroke

When to talk with your doctor

Addressing these, such as treating hearing or vision changes and staying connected with others, may help support brain health over time.

Support, education and connection with others can help as you adjust and find ways forward. To learn more, read the "Advanced Thinking Changes" fact sheet at [Parkinson.org/Library](https://parkinson.org/Library) or watch the webinar "Dementia Support Beyond Medications" at [Parkinson.org/YouTube](https://parkinson.org/YouTube).

Hallucinations or Delusions

Some people with Parkinson's experience hallucinations (seeing, hearing or sensing things that are not there) or delusions (strong beliefs that do not match reality). These symptoms may be related to Parkinson's, thinking changes, medications, illness or infection.

Because hallucinations and delusions can affect safety, daily life and relationships, it is important to discuss changes with the care team.

What can help

- Keep rooms well lit and reduce shadows, clutter or background noise
- Keep surroundings and daily routines as familiar as possible
- Review medications with the care team

Care Partner Tip

Stay calm and offer reassurance. Avoid questioning what the person is seeing or believing. Instead, acknowledge their experience ("I understand that's what it seems like") and gently redirect to a calming activity or another part of the room.

When to Talk with Your Care Team

Talk with your care team if you notice new thinking changes, changes that worsen or anything that affects safety or daily life. Sudden confusion or rapid changes need immediate medical attention, as they may be caused by illness, dehydration, medication side effects or other treatable conditions.

When to talk with your doctor

Different health professionals can help with thinking changes and offer practical strategies for everyday tasks and routines.

Neuropsychologists

Evaluate thinking and memory through detailed testing. The results can help explain changes in attention, memory or problem-solving and guide treatment and support.

Psychiatrists

Assess and treat mood, anxiety or behavioral changes that can affect thinking. They can also review and adjust medications.

Occupational Therapists

Suggest ways to organize routines, break tasks into steps or adjust the home environment (such as lighting, layout or reducing clutter) to improve safety and make activities easier and less stressful.

Speech Therapists (also called speech-language pathologists)

Help with communication and thinking skills. They may teach strategies to find words, stay focused, remember information or organize thoughts during conversations and daily activities.

Physical Therapists

Design exercise programs that support movement, balance and brain health. Regular physical activity may also support attention, thinking and overall well-being.

Helpful Resources

- Read the book “Cognition: A Guide to Thinking Changes” at [Parkinson.org/Cognition](https://parkinson.org/Cognition).
- Learn more about thinking changes, dementia, hallucinations and delusions in Parkinson’s at [Parkinson.org/NonMovement](https://parkinson.org/NonMovement).
- Watch the CareMAP videos on advanced thinking changes for practical tools and tips designed for care partners at [Parkinson.org/CareMAP](https://parkinson.org/CareMAP).



How to Add Palliative Care to Your Parkinson's Plan

Palliative care is extra support for anyone living with a serious illness, such as Parkinson's. It helps manage symptoms and supports emotional well-being. The goal is to improve quality of life for you and your care partners.

Palliative care can help you:

 Manage difficult symptoms such as pain, anxiety and constipation	 Cope with the stress and emotional impact of illness	 Support your care partner with education and counseling	 Plan for future care and make decisions as your needs change
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How does palliative care work?

Palliative care can be part of your regular care. In some settings, it involves a specialized team that may include nurses, social workers, chaplains and other health professionals. It often starts with a conversation about what matters most to you, followed by a plan to address those needs. It does not replace your current treatment.

When should I get palliative care?

Palliative care can be helpful at any stage of Parkinson's, even soon after diagnosis. You might consider palliative care if symptoms are hard to manage, daily life feels overwhelming or you want help planning ahead. It is not limited to hospice or end-of-life care.

How can I get palliative care?

- **Ask about palliative care at your next medical visit.** If you are in the hospital, in rehabilitation or receiving home health services, ask if palliative care is available.
- **Visit www.getpalliativecare.org to find services near you.** Many palliative care agencies offer support at any stage of illness.

Will my insurance cover palliative care?

Medicare typically covers many palliative care services. Coverage through Medicaid and private insurance depends on your state and your plan. Check with your insurance company.

Visit Parkinson.org/PalliativeCare for practical tips and resources.

Parkinson's Foundation Resources



Helpline

Our Helpline specialists support people living with PD and care partners. Get answers and connect with local resources. Call **1-800-4PD-INFO (1-800-473-4636)** or email **Helpline@Parkinson.org**.



PD Library

Explore our books, fact sheets, webinars, podcasts and more in our PD library. Select topic “advanced PD” for resources to support advancing needs. Visit **Parkinson.org/Library**.



PD Health @ Home

Discover PD Health @ Home virtual programs in English and Spanish — Mindfulness Mondays, Wellness Wednesdays and Fitness Fridays. Visit **Parkinson.org/PDHealth**.

Key resources

Webinars

Visit **Parkinson.org/YouTube** and search the following titles:

- Exploring Home Safety Modifications for Aging in Place
- Tips for Daily Living with Parkinson's Disease: Easing Movement Challenges
- Optimizing Your Quality of Life with Parkinson's
- Expert Briefing: Living Alone: Home Safety & Management
- Expert Briefing: Advanced Parkinson's and Palliative Care in the 21st Century

Podcasts

Visit **Parkinson.org/Library** and search the following titles:

- Aging in Place
- Palliative Care as Supportive Care in PD
- Allied Health Spotlight — A series of podcasts featuring occupational, physical and speech therapies

Webpages

- **Parkinson.org/HomeSafety**
- **Parkinson.org/AssistiveDevices**
- **Parkinson.org/Therapies**

Parkinson's Foundation Care Partner Resources



Care Partner Guide

Find practical tips for self-care, support, daily needs, relationships and future planning at [Parkinson.org/CarePartnerGuide](https://parkinson.org/CarePartnerGuide).



CareMAP Videos

Watch how-to videos to get practical tips and hear stories from care partners at [Parkinson.org/CareMAP](https://parkinson.org/CareMAP).



Care Partner Webpages

Find practical tips and support for care partners at every stage at [Parkinson.org/CarePartners](https://parkinson.org/CarePartners).

Key resources

Webinars

Visit [Parkinson.org/YouTube](https://parkinson.org/YouTube) and search the following titles:

- Care Partner Conversations
- Care Partner Summit
- Male Care Partner Webinar
- Exploring Next Steps in Care

Podcasts

Visit [Parkinson.org/Library](https://parkinson.org/Library) and search the following titles:

- The Care Partner Experience
- Caregiving Over the Years
- The Role of Caregivers in Parkinson's Dementia

Webpages

- [Parkinson.org/AdvancedPD](https://parkinson.org/AdvancedPD)
- [Parkinson.org/OutsideHelp](https://parkinson.org/OutsideHelp)
- [Parkinson.org/SelfCare](https://parkinson.org/SelfCare)

Self-paced online course

- [Parkinson.org/CarePartnerProgram](https://parkinson.org/CarePartnerProgram)

Additional Resources

AARP

Offers tips for making your home safer and easier to move around in, resources for caregivers and information about planning for future care. Resources include the AARP HomeFit Guide, free guide with room-by-room home safety ideas. Visit [AARP.org](https://www.aarp.org) or [AARP.org/HomeFit](https://www.aarp.org/HomeFit).

ARCH Respite Locator

A tool for finding respite care services that offer a break for the primary care partner. Respite may include in-home or out-of-home services and may last from a few hours to overnight. Visit [ARCHRespite.org](https://www.archrespite.org).

Area Agency on Agencies

Local agencies that connect people with community programs and services, such as transportation, meal programs and caregiver support. Call 1-800-677-1116 or visit [Eldercare.acl.gov](https://www.eldercare.acl.gov).

Association for Driver Rehabilitation Specialists

Find professionals who offer driving evaluations and additional driving resources. Visit [Aded.net](https://www.aded.net).

Center for Disease Control and Prevention

Offers information about preventing falls and responding to a fall. Visit [CDC.gov](https://www.cdc.gov) and search “fall prevention.” You can also visit [LifeFallPrevention.com](https://www.life-fall-prevention.com), a CDC Foundation resource on fall prevention.

Meals on Wheels

A nationwide program that delivers meals to older adults and people with disabilities who cannot shop or cook. Many programs also include a friendly check-in from volunteers. Visit [Mealsonwheelsamerica.org](https://www.mealsonwheelsamerica.org).

National Council on Aging

Shares resources and tools related to transportation, fall prevention, medical alert systems, financial planning and senior centers. Visit [Ncoa.org](https://www.ncoa.org).

National Institute on Aging

Offers information on healthy aging, caregiving, nutrition, exercise, fall prevention and safety. Visit [Nia.nih.gov](https://www.nia.nih.gov).

About the Parkinson's Foundation

The Parkinson's Foundation makes life better for people with Parkinson's disease by improving care and advancing research toward a cure. In everything we do, we build on the energy, experience and passion of our global Parkinson's community. A wealth of information about Parkinson's, events and resources are available at [Parkinson.org](https://www.parkinson.org).

Your generosity makes this publication possible.

The Parkinson's Foundation is proud to provide this booklet and other educational materials at no cost to people around the globe. If you found this book helpful, please consider a gift so that we may continue to fight Parkinson's on all fronts: funding innovative research, providing support services and offering educational materials such as this publication.

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5757 Waterford District Drive
Miami, FL 33126

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WWW.PARKINSON.ORG

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